



Reporting of Waiting Lists for Mental Health Services

**As Required by
2024-2025 General Appropriations Act
House Bill (H.B.) 1, 88th Legislature,
Regular Session, 2023 (Article II,
Health and Human Services
Commission, Rider 45)**

**Texas Health and Human Services
May 2025**



TEXAS
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Table of Contents

1. Introduction	2
Data Context and Limitations.....	2
2. Adult and Children’s Community Mental Health Services Waiting List Data	4
Adult Community Mental Health Services.....	4
Children’s Community Mental Health Services	7
Funds Expended to Remove People from Waiting Lists	9
Factors Affecting Community Mental Health Waiting Lists for Adults and Children	9
3. Forensic Inpatient Waiting List Data	11
Factors Impacting Forensic Inpatient Waiting Lists	14
Funds Expended to Remove People from Forensic Inpatient Bed Waiting Lists...	19
List of Acronyms	22

1. Introduction

The *Reporting of Waiting Lists for Mental Health Services* meets the requirements of the 2024-25 General Appropriations Act, House Bill (H.B.) 1, 88th Legislature, Regular Session, 2023 (Article II, Health and Human Services Commission, Rider 45). Rider 45 requires Health and Human Services Commission (HHSC) to report on waiting lists for mental health services to the Legislative Budget Board and Governor by November 1 and May 1 of each fiscal year.

Rider 45 requires the report to include waiting list and expenditure data for community mental health services for adults and children, forensic state hospital beds, and maximum-security forensic state hospital beds for each local mental health authority (LMHA), local behavioral health authority (LBHA), state facility, and contracted entity. Required data include:

- Number of people waiting for all services;
- Number of underserved¹ people waiting for additional services;
- Number of people removed from the waiting list;
- Funds expended to remove people on the waiting list in each fiscal quarter included in the reporting period;
- Average number of days spent on a waiting list; and
- Explanation whether waiting lists are due to operational or other short-term or long-term factors, due to insufficient capacity.

The reporting period for this report is fiscal year 2025, quarters one and two.

Data Context and Limitations

The LMHA and LBHA community mental health waiting lists are point-in-time² calculations of the total number of people waiting for mental health services at the end of the respective quarter of the reporting period. The waiting lists are subject to daily fluctuations as people are added or removed for various reasons (e.g.,

¹ Term used to designate persons who qualified for a higher level of care, based on assessment scores, but received a lower level of care due to staff or resource limitations.

² Calculated at the end of the fiscal quarter.

admitted to services, no longer interested in services, receiving services elsewhere, no longer able to be contacted).

As part of the federal response to the coronavirus disease 2019 (COVID-19) pandemic, the Substance Abuse and Mental Health Services Administration awarded HHSC supplemental block grant funding, part of which HHSC allocated to LMHAs and LBHAs to increase the number of people served in fiscal years 2021-2024. The COVID-19 pandemic affected many providers' ability to attract and retain a stable workforce, both during and after the pandemic, resulting in unprecedented workforce shortages statewide. This can result in a persistent population of underserved individuals due to lack of staff at LMHAs and LBHAs.

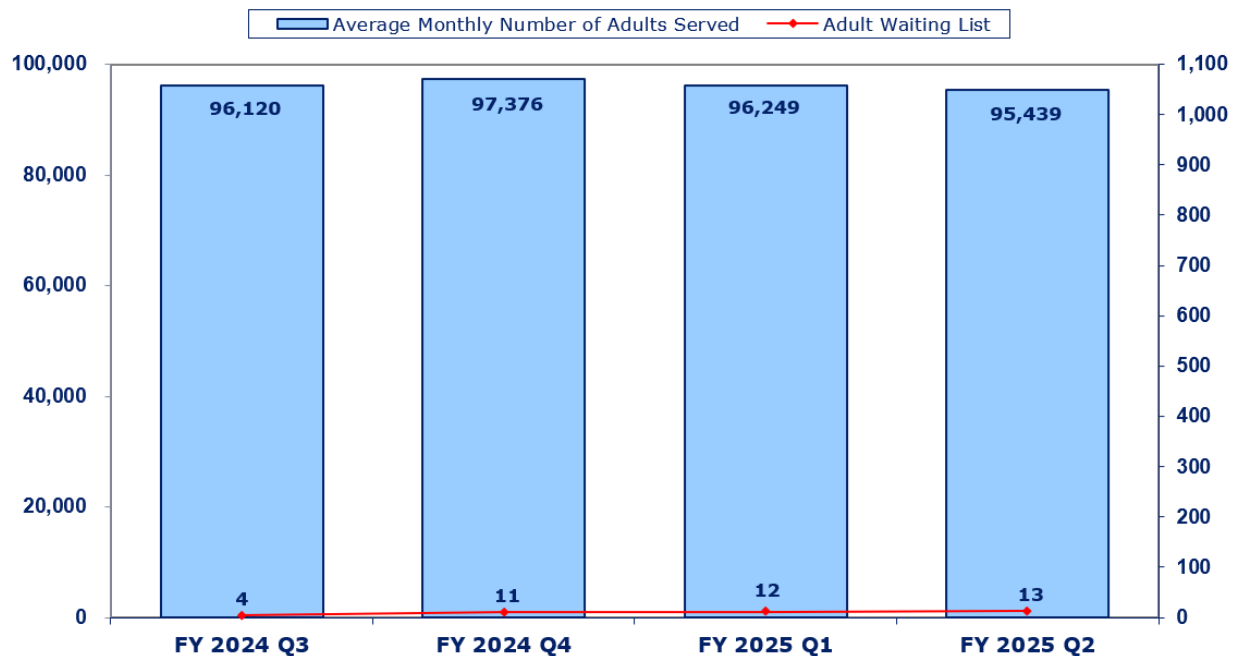
If the demand for services increases beyond the total funded capacity for community mental health services and available staff resources, a waiting list may remain or increase despite the effective use of additional funding and an LMHA's or LBHA's success in meeting its contracted targets. It is anticipated that waitlists may begin to increase due to the discontinuation of the COVID-19 related federal block grant funding on March 24, 2025 and/or there will be an increase in numbers of persons underserved.

2. Adult and Children's Community Mental Health Services Waiting List Data

Adult Community Mental Health Services

Figure 1 shows the average monthly number of adults receiving community mental health services and the number of adults on the waiting list at the end of the fiscal quarter.³

Figure 1. Adult Community Mental Health Number Served and Waiting List for FY 2024 (Q3-Q4) and FY 2025 (Q1-Q2) ⁴



³ Source: Client Assignment and Registration, March 2025.

⁴ "Average Monthly Number of Adults Served" uses Legislative Budget Board performance measure D.2.1. Strategy: Community Mental Health Svcs-Adults – Average Monthly Number of Adults Receiving Community Mental Health Services.

Table 1. Adult Community Mental Health Number Served and Waiting List for FY 2024 (Q3-Q4) and FY 2025 (Q1-Q2)

Category	FY 2024 Q3	FY 2024 Q4	FY 2025 Q1	FY 2025 Q2
Average Monthly Number of Adults Served	96,120	97,376	96,249	95,439
Adult Waiting List	4	11	12	13

Table 2 and Table 3 outline the required report elements for the community mental health services for adults in LMHAs and LBHAs with waiting lists.

Table 2. Number of Adults Waiting for All Services or Removed from a Waiting List and Average Number of Days on a Waiting List by LMHA or LBHA⁵

LMHA or LBHA	No. Waiting for All Services FY 25 Q1	No. Waiting for All Services FY 25 Q2	No. Removed from Waiting List FY 25 Q1	No. Removed from Waiting List FY 25 Q2	Average No. of Days on Waiting List FY 25 Q1	Average No. of Days on Waiting List FY 25 Q2
Gulf Bend Center	12	13	1	3	100	120
Total	12	13	1	3	100	120

Table 3. Number of Underserved⁶ Adults Waiting for Additional Services by LMHA or LBHA⁷

LMHA or LBHA	FY 25 Q1	FY 25 Q2
Anderson Cherokee Community Enrichment Services (ACCESS)	1	2
Andrews Center Behavioral Healthcare System	53	38
Austin Travis County Integral Care	286	265
Betty Hardwick Center	1	4
Border Region Behavioral Health Center	26	23
Camino Real Community Services	22	16
Center for Health Care Services	38	36
Center for Life Resources	11	8

⁵ LMHAs and LBHAs with no underserved waiting lists from FY 25 Q1 to FY 25 Q2 are excluded.

⁶ Term used to designate persons who qualified for a higher level of care, based on assessment scores, but received a lower level of care due to staff or resource limitations.

⁷ Data does not include adults in a lower level of care due to "consumer refused."

LMHA or LBHA	FY 25 Q1	FY 25 Q2
Central Plains Center	1	1
Denton County MHMR Center	32	40
Emergence Health Network	29	27
Gulf Bend Center	12	8
Gulf Coast Center	2	1
Helen Farabee Centers	0	1
MHMR Authority of Brazos Valley	51	46
Nueces Center for Mental Health and Intellectual Disabilities	93	61
Pecan Valley Centers for Behavioral and Developmental Healthcare	132	98
PermiaCare	1	1
Community Healthcore	41	30
Spindletop Center	86	47
StarCare Specialty Health System	333	284
Texas Panhandle Centers	3	1
Texoma Community Center	2	3
Tri-County Behavioral Healthcare	55	53
Tropical Texas Behavioral Health	216	196
West Texas Centers	1	1
Total	1,528	1,291

Children’s Community Mental Health Services

The following figure shows the average monthly number of children receiving community mental health services and the number of children waiting for community mental health services at the end of the fiscal quarter.⁸

Figure 2. Children’s Community Mental Health Number Served and Waiting List for FY 24 (Q3-Q4) and FY 25 (Q1-Q2) ⁹

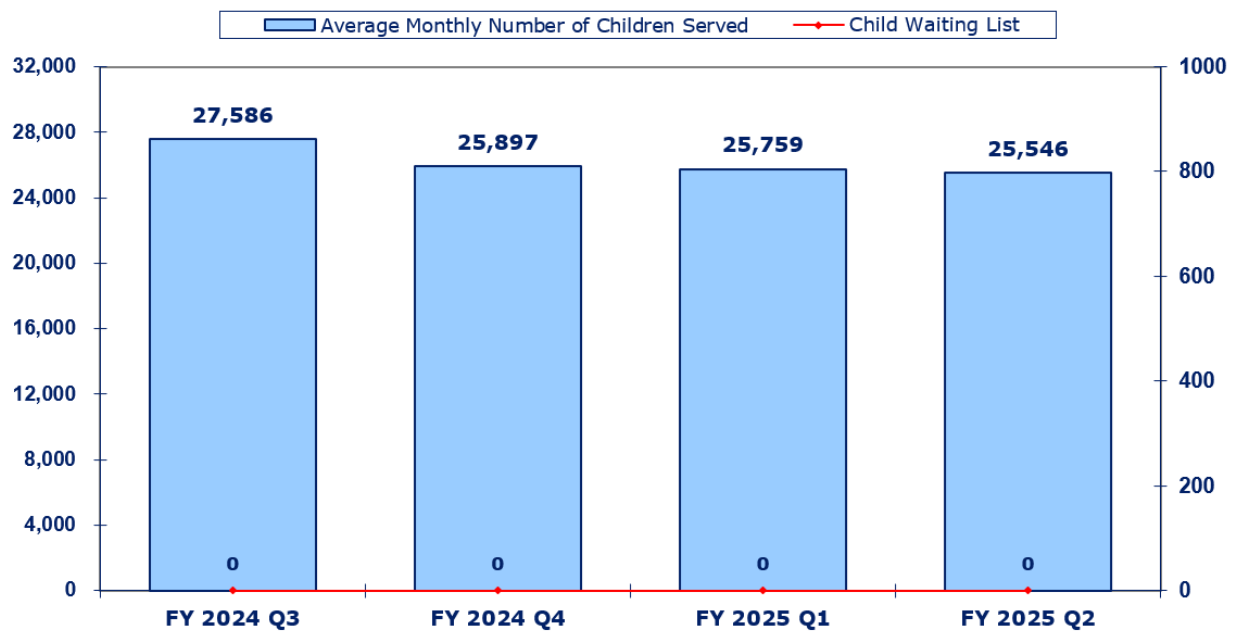


Table 4. Children’s Community Mental Health Number Served and Waiting List for FY 24 (Q3-Q4) and FY 25 (Q1-Q2)

Category	FY 24 Q3	FY 24 Q4	FY 25 Q1	FY 25 Q2
Average Monthly Number of Children Served	27,586	25,897	25,759	25,546

⁸ Source: Client Assignment and Registration, March 2025.

⁹ “Average Monthly Number of Children Served” uses Legislative Budget Board performance measure D.2.2 Strategy: Community Mental Health Services-Children – Average Monthly Number of Children Receiving Community Mental Health Services.

Category	FY 24 Q3	FY 24 Q4	FY 25 Q1	FY 25 Q2
Child Waiting List	0	0	0	0

Table 5 and Table 6 summarize the required report elements for the community mental health services waiting list for children.

Table 5. Number of Children Waiting for All Services or Removed from a Waiting List and Average Number of Days on a Waiting List by LMHA or LBHA

LMHA or LBHA	No. Waiting for All Services FY 25 Q1	No. Waiting for All Services FY 25 Q2	No. Removed from Waiting List FY 25 Q1	No. Removed from Waiting List FY 25 Q2	Average No. of Days on Waiting List FY 25 Q1	Average No. of Days on Waiting List FY 25 Q2
Total	0	0	0	0	0	0

Table 6. Number of Underserved¹⁰ Children Waiting for Additional Services by LMHA or LBHA^{11,12}

LMHA or LBHA	FY 25 Q1	FY 25 Q2
Anderson Cherokee Community Enrichment Services (ACCESS)	0	1
Andrews Center Behavioral Healthcare System	4	1
Austin Travis County Integral Care	1	0
Border Region Behavioral Health Center	0	1
Central Counties Center for MHMR Services	0	1
Community Healthcore	107	105
Denton County MHMR Center	4	0
Emergence Health Network	73	4
Heart of Texas Behavioral Health Network	1	43
Lakes Regional Community Center	1	0
Nueces Center for Mental Health and Intellectual Disabilities	1	0
Pecan Valley Centers for Behavioral & Developmental Healthcare	26	10

¹⁰ Term used to designate persons who qualified for a higher level of care, based on assessment scores, but received a lower level of care due to staff or resource limitations.

¹¹ Data does not include children in a lower level of care due to "consumer refused."

¹² LMHAs and LBHAs with no underserved waiting lists from FY 25 Q1 to Q2 are excluded.

LMHA or LBHA	FY 25 Q1	FY 25 Q2
PermiaCare	0	1
Spindletop Center	203	42
Texas Panhandle Centers	0	1
Texoma Community Center	3	0
Tri-County Behavioral Healthcare	0	2
Tropical Texas Behavioral Health	30	19
West Texas Centers	1	3
Total	455	234

Funds Expended to Remove People from Waiting Lists

The 2024-25 General Appropriations Act, H.B. 1, 88th Legislature, Regular Session, 2023, appropriated \$451,244,249 per fiscal year for adult community mental health services (D.2.1 Strategy) and \$110,629,159 per fiscal year for children’s community mental health services (D.2.2 Strategy). HHSC’s Utilization Management Program Manual requires LMHAs and LBHAs to prioritize the reduction of the waiting list as soon as service capacity allows.

In accordance with the federal HR-133 Coronavirus Response and Relief Supplemental Appropriations Act, 2021, and the American Rescue Plan Act, 2021, the Substance Abuse and Mental Health Services Administration awarded Texas a combined total of \$203.4 million in Community Mental Health Services Block Grant supplemental funds, of which HHSC allocated \$78.2 million toward the Mental Health Outpatient Capacity Expansion project. LMHAs and LBHAs continued expending the funds in the fiscal year quarters addressed in this report to address workforce shortages in order to serve additional people.

Factors Affecting Community Mental Health Waiting Lists for Adults and Children

LMHAs and LBHAs with waiting lists report ongoing workforce shortage challenges, including difficulties in recruiting and retaining behavioral health providers, particularly Licensed Practitioners of the Healing Arts and Qualified Mental Health Professionals. These challenges stem from a competitive job market in the mental healthcare industry and opportunities from other employers statewide. LMHAs and LBHAs had limited funding to retain existing staff or increase targeted recruiting

efforts. HHSC is currently working to expand the mental health workforce by updating the Qualified Mental Health Professional credential in the Texas Administrative Code.

LMHAs and LBHAs across the state face professional mental health workforce shortages, with the impact on community mental health waiting lists being greatest for rural LMHAs. Gulf Bend Center, the one LMHA that reported a waiting list for adults, is a rural provider in a designated health professional shortage area. However, Gulf Bend Center surpassed its contracted service targets every month in the first two quarters of fiscal year 2025 (maximum of 103% in September) with the exception of the previous two service months, serving 95% in January and 98% in February.

To minimize the adult and child waiting lists, HHSC monitored the community mental health waiting list monthly and contacted LMHAs and LBHAs reporting waiting lists to obtain details on the causes and efforts taken to reduce them. HHSC provided technical assistance to these LMHAs and LBHAs by addressing workforce needs and offering guidance for expanding service capacity.

3. Forensic Inpatient Waiting List Data

Tables 7-12 below show the number of people waiting, number of people removed, and average number of days spent waiting on the forensic inpatient bed waiting lists for the first two quarters of fiscal year 2025. HHSC's Health and Specialty Care System (HSCS) maintains two distinct lists of pending admissions for people on inpatient forensic commitments: one for people pending admission to a non-maximum-security unit (non-MSU); and one for people pending admission to a maximum-security unit (MSU).

Individuals may be admitted to a different facility than the facility they are noted to be waiting for. This may be due to availability of special population beds, admission to a contract bed with a statewide catchment area, or other factors. Additionally, individuals waiting for admission to an MSU are not added to a facility-specific waiting list until closer to their admission because all MSUs serve a statewide catchment area, rather than a region-specific catchment area.

The number of people on the waitlist is a snapshot of the last business day of the quarter. The number of people removed from a forensic inpatient bed waiting list includes people removed due to admission, case resolution, restoration of competency in jail, and various other dispositions.

The average number of days spent on a forensic inpatient bed waiting list is measured from the date the court notifies HSCS of the commitment to the date the person is admitted to the hospital. If there is not an average number of days noted in the tables below, that means no individuals were admitted to the facility from the waitlist during that quarter.

Table 7. Number of People on Non-MSU Forensic Inpatient Waiting List

Facility/Contractor	FY 25 Q1	FY 25 Q2
Austin State Hospital	197	214
Big Spring State Hospital	31	25
Dallas Behavioral Hospital	1	0
El Paso Psychiatric Center	68	58
Harris County Psychiatric Center	0	0
Integral Care	5	3
John S. Dunn Behavioral Center	175	183
Kerrville State Hospital	2	2
Montgomery County Hospital	13	16
North Texas Behavioral Health Authority	17	6
North Texas State Hospital Wichita Falls	176	169
Palestine Regional	3	16
Perimeter Behavioral Hospital	0	0
Rio Grande State Center	0	0
Rusk State Hospital	110	112
San Antonio State Hospital	209	176
Terrell State Hospital	240	215
Total	1,247	1,195

Table 8. Number of People Removed from Non-MSU Forensic Inpatient Waiting List

Facility/Contractor	FY 25 Q1	FY 25 Q2
Austin State Hospital	69	62
Big Spring State Hospital	43	61
Dallas Behavioral Hospital	4	0
El Paso Psychiatric Center	19	18
Harris County Psychiatric Center	11	16
Integral Care	51	40
John S. Dunn Behavioral Center	108	128
Kerrville State Hospital	6	7
Montgomery County Hospital	64	39
North Texas Behavioral Health Authority	14	32
North Texas State Hospital Wichita Falls	85	79
Palestine Regional	40	32
Perimeter Behavioral Hospital	7	17
Rio Grande State Center	0	2

Facility/Contractor	FY 25 Q1	FY 25 Q2
Rusk State Hospital	15	8
San Antonio State Hospital	71	71
Terrell State Hospital	74	60
Total	681	672

Table 9. Average Number of Days on Non-MSU Forensic Inpatient Waiting List

Facility/Contractor	FY 25 Q1	FY 25 Q2
Austin State Hospital	84.3	165.3
Big Spring State Hospital	123.8	148.9
Dallas Behavioral Hospital	205.5	-
El Paso Psychiatric Center	316.9	494.7
Harris County Psychiatric Center	104.6	92.3
Integral Care	182.3	155
John S. Dunn Behavioral Sciences Center	138.3	136.6
Kerrville State Hospital	34.2	45.7
Montgomery County Hospital	307.1	303.1
North Texas Behavioral Health Authority	249.8	227.7
North Texas State Hospital Wichita Falls	150.1	183.3
Palestine Regional	248.3	250.5
Perimeter Behavioral Hospital	258.3	201.9
Rio Grande State Center	-	779.5
Rusk State Hospital	592.3	340.7
San Antonio State Hospital	236.3	333.6
Terrell State Hospital	210.6	204.7
Total	197.6	198.1

Table 10. Number of People on MSU Forensic Inpatient Waiting List

Facility	FY 25 Q1	FY 25 Q2
Perimeter Behavioral Hospital	12	8
Rio Grande State Center	464	423
Rusk State Hospital	36	56
San Antonio State Hospital	2	7
Terrell State Hospital	9	11
Total	523	505

Table 11. Number of People Removed from MSU Forensic Inpatient Waiting List

Facility	FY 25 Q1	FY 25 Q2
Perimeter Behavioral Hospital	41	29
Rio Grande State Center	62	46
Rusk State Hospital	84	97
San Antonio State Hospital	20	18
Terrell State Hospital	49	44
Total	256	234

Table 12. Average Number of Days on MSU Forensic Inpatient Waiting List

Facility	FY 25 Q1	FY 25 Q2
Perimeter Behavioral Hospital	253.1	172.7
Rio Grande State Center	-	-
Rusk State Hospital	338.4	287
San Antonio State Hospital	287.5	209.9
Terrell State Hospital	287.9	222.3
Total	302.3	246.6

Factors Impacting Forensic Inpatient Waiting Lists

Below are the long-term factors that impact the forensic waiting list. Currently, there are no short-term factors impacting the forensic waiting list.

Deferred Maintenance and Emergency Repairs

State hospitals are required to comply with the federal Life Safety Code and regulatory requirements such as ensuring the mitigation of ligature risks.¹³ State hospital beds are taken offline when mitigation and remediation efforts or maintenance are required.

The Supplemental Appropriations Act, Senate Bill (S.B.) 30, 88th Legislature, Regular Session, 2023, appropriated \$50,000,000 for deferred maintenance and

¹³ The Centers for Medicare and Medicaid Services notes that a ligature risk (point) may include anything which could be used to create a sustainable attachment point such as a cord, rope, or other material for the purpose of hanging or strangulation.

\$14,000,000 for emergency repairs to Strategy G.4.2, Facility Repairs and Renovation, to be used for state hospitals and state supported living centers. Currently the cost of anti-ligature remediation and other necessary repairs for the state hospitals exceeds what has been appropriated and the state hospitals experience challenges maintaining buildings with aging infrastructure and building systems, which further impacts the number of patients who can be served. HSCS has a deferred maintenance need of approximately \$800,000,000 and requests appropriations each legislative session to maintain a consistent cycle of projects to meet this need.

County Planning, Coordination, and Education

Planning, coordination, and education among county stakeholders can impact the demand for inpatient competency restoration services. Collaboration with stakeholders can help to address effective and timely services depending on the individual's needs and placements, such as when an individual's competency to stand trial is restored prior to admission to a state hospital, or when a person is not likely to restore to competency in the foreseeable future but may be able to successfully live and receive treatment in the community. Education on the competency restoration process and its appropriate use can also reduce the use of inpatient competency restoration and increase the use of outpatient and jail-based competency restoration programs and alternative dispositions. The HSCS Office of Forensic Services and Coordination (OFSC) proactively reaches out to stakeholders to provide education and enhance collaboration with counties.

Additionally, OFSC engages counties through the Jail In-Reach Program which aims to educate and collaborate with external stakeholders and community-based teams to support active forensic waitlist monitoring and services through:

- Clinical consultation services that may assist with psychiatric stabilization;
- Trial competency re-evaluations in the event of immediate restoration while awaiting state hospital transfer;
- Legal education on options for alternative case dispositions; and
- Enhanced follow-ups on patients restored to competency and returned to jail to prevent clinical decompensation and unnecessary rehospitalization.

The OFSC and Behavioral Health Services Forensic and Jail Diversion Services promote statewide initiatives that support local communities in behavioral health and justice planning to reduce and prevent justice-involvement for people with

behavioral health needs, with the downstream impact of reducing the number of people in the competency restoration process. OFSC and Forensic and Jail Diversion Services have conducted local strategic planning with counties through Sequential Intercept Model Mapping Workshops.

Sequential Intercept Model Mapping Workshops support counties in developing strategic plans that increase the use of diversion and reduce reliance on inpatient competency restoration services. HHSC plans to continue and expand community-based Sequential Intercept Model mapping exercises in FY26 to support local jurisdictions in reducing their need for inpatient competency restoration.

OFSC, in collaboration with the Texas Judicial Commission on Mental Health launched the Eliminate the Wait Campaign in 2021, which provides municipal and county behavioral health and justice stakeholders with training and technical assistance on strategies that reduce reliance on inpatient competency restoration services. A [toolkit](#) is publicly available and trainings are provided on request.

The Texas Behavioral Health and Justice Technical Assistance Center provides statewide training and technical assistance on competency restoration processes, diversion, and other issues that impact forensics and justice.

Current Construction

HSCS is ensuring current and future state hospital capacity through the following projects:

San Antonio State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$15,000,000 to Strategy G.4.2, Facility Repairs and Renovation, to convert the 40-bed Alamo Hall at San Antonio State Hospital to an MSU, and HHSC is designating this unit to serve the geriatric MSU population. The architecture and engineering (A/E) contract was executed in August 2023, and the construction contract is anticipated to be awarded in spring 2025. Construction completion is estimated for spring 2026.

Dallas State Hospital

The Dallas State Hospital will be a new 292-bed inpatient psychiatric hospital with 200 adult beds, 75 of which will serve the forensic population, and 92 pediatric beds. Both units are under construction with completion estimated for July 2025 for the adult unit, and April 2026 for the pediatric unit.

North Texas State Hospital – Wichita Falls Campus

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$452,000,000 to Strategy G.4.2 to construct a 200-bed replacement hospital for the North Texas State Hospital - Wichita Falls campus to include 24 MSU beds, 16 civil beds, 24 adolescent beds, and 136 non-MSU forensic beds. The A/E contract was executed in December 2023, and the construction manager at risk (CMR) contract was executed in May 2024. The groundbreaking ceremony took place on September 24, 2024. Construction completion is estimated for summer 2027.

Terrell State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$573,000,000 to Strategy G.4.2 to construct a 250-bed replacement hospital for the Terrell State Hospital campus to include 50 MSU beds, 25 civil beds, 35 adolescent beds, and 140 non-MSU forensic beds. The A/E contract was executed in January 2024, and the CMR contract was executed in July 2024. The groundbreaking ceremony took place on August 28, 2024. Construction completion is estimated for summer 2027.

Rio Grande State Center

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$120,000,000 to Strategy G.4.2 to expand the Rio Grande State Center by 50 MSU beds. The A/E contract was executed in February 2024, and the CMR contract was executed in August 2024. Construction is estimated to begin in spring 2025 and reach substantial completion in spring 2027.

El Paso Psychiatric Center

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$50,000,000 to Strategy G.4.2 for land acquisition, pre-planning, planning, and initial construction of a 50-bed expansion of El Paso Psychiatric Center, to include 25 non-MSU forensic beds. The A/E contract was executed in March 2024, and land acquisition is in progress. A construction timeline will depend on land acquisition and final legislative appropriations to complete construction.

Panhandle State Hospital

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$159,000,000 to Strategy G.4.2 to construct a new 75-bed hospital in Amarillo to include 50 non-MSU forensic beds. The A/E contract was executed in December 2023, and the CMR contract was executed in July 2024. The land lease agreement was executed in

January 2025. HHSC estimates construction will begin in spring 2025 and reach substantial completion in spring 2027.

Lubbock Psychiatric Center

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$121,000,000 to Strategy G.2.4 to construct a new 50-bed MSU hospital in Lubbock. The land lease agreement between TDCJ and HHSC is nearing completion. The A/E contract was executed in February 2024, and the CMR contract was executed in July 2024. HHSC estimates construction will begin spring 2025 and reach substantial completion in March 2027.

Additional Capacity

The General Appropriations Act, H.B. 1, 88th Legislature, Regular Session, 2023 (Article II, HHSC, Rider 52), appropriated \$45,834,616 per fiscal year to Strategy G.2.2., Community Mental Health Hospitals, to contract for 150 additional competency restoration beds. As of fiscal year 2025, quarter three, contracts are executed for 113 of the 150 beds and HHSC intends to have all operational by the end of the fiscal year. The General Appropriations Act, H.B. 1, 88th Legislature, Regular Session, 2023 (Article II, HHSC, Rider 52), appropriated \$4,068,000 per fiscal year to Strategy G.2.2., Community Mental Health Hospitals, to contract for 16 additional beds. This contract was awarded to Perimeter Behavioral Hospital who is now providing competency restoration services to the geriatric population.

S.B. 30, 88th Legislature, Regular Session, 2023, appropriated \$239,000,000 in grants to Strategy G.4.2 for construction of up to 332 inpatient beds, half of which must serve forensic patients, in the Rio Grande Valley, Victoria County, Montgomery County, and City of Beaumont areas. All grant award contracts have been executed.

Unscheduled Admissions

A growing number of unscheduled admissions or “walk-ins” impact admissions to state hospitals. Walk-ins are typically referrals primarily from law enforcement, family members, or the individual themselves appearing at a state hospital for the purpose of admission and may be either admitted or denied (Table 13). State hospitals must comply with the [Emergency Medical Treatment and Active Labor Act](#). This means if a person needs immediate crisis stabilization, state hospitals must assess the individual and provide stabilizing care if possible or transfer that person to another facility for treatment with that facility's agreement.

Table 13. Total Walk-ins (Admitted and Denied)

Facility	FY 25 Q1	FY 25 Q2
Austin State Hospital	39	32
Big Spring State Hospital	1	4
El Paso Psychiatric Center	115	121
Kerrville State Hospital	0	0
North Texas State Hospital	0	0
Rio Grande State Center	0	0
Rusk State Hospital	5	8
San Antonio State Hospital	34	54
Terrell State Hospital	378	357
Total	572	576

Beds occupied by walk-in admissions limit the number of beds available for planned admissions from the civil waitlist, which are typically coordinated through the LMHA or LBHA, and forensic waitlists, which are typically coordinated with the designated party in the county of an individual's commitment (Table 14).

Table 14. Walk-in Admissions

Facility	FY 25 Q1	FY 25 Q2
Austin State Hospital	13	17
Big Spring State Hospital	0	0
El Paso Psychiatric Center	56	55
Kerrville State Hospital	0	0
North Texas State Hospital	0	0
Rio Grande State Center	0	0
Rusk State Hospital	4	5
San Antonio State Hospital	21	33
Terrell State Hospital	300	251
Total	394	361

Funds Expended to Remove People from Forensic Inpatient Bed Waiting Lists

Table 15, below, details the funds expended to remove people from the forensic waiting lists in each quarter of fiscal year 2024. Please note that the expenditure data represents spending on a cash basis by fiscal quarter for any open appropriation year.

State hospital operations includes all expenditures from Strategy G.2.1, Mental Health State Hospitals, including funds used for salaries, supplies, other items, and excluding funds used for contract beds. State hospital operations expenditures include funds used to serve youth, and civilly committed and voluntarily admitted adult patients at state hospitals in addition to forensic patients.

State hospital contract beds includes expenditures from Strategy G.2.1, Mental Health State Hospitals, and Strategy G.2.2, Community Mental Health Hospitals, that fund the contracts under HSCS for the operation of beds at the John S. Dunn Behavioral Sciences Center, Montgomery County Mental Health Treatment Facility, Palestine Regional Medical Center, University of Texas Health Science Center at Tyler, Perimeter Behavioral Hospital, and additional beds subcontracted at facilities through HHSC contracts with North Texas Behavioral Health Authority and Integral Care. State hospital contract beds expenditures include funds used to serve civil patients at John S. Dunn Behavioral Sciences Center and University of Texas Health Science Center at Tyler in addition to forensic patients.

State hospital deferred maintenance and state hospital emergency repairs include expenditures from Strategy G.4.2. Additional deferred maintenance and emergency repair expenditures may be included within the state hospital operations category if funded within a state hospital's existing budget.

Table 15. Forensic Waiting Lists Related Expenditures

Expenditures	FY 25 Q1	FY 25 Q2	TOTAL FYTD 25
State Hospital Operations	\$151,092,132	\$164,051,442	\$315,143,573
State Hospital Contract Beds	\$15,800,685	\$21,836,529	\$37,637,214
State Hospital Construction	\$56,120,382	\$150,251,227	\$206,371,609
State Hospital Deferred Maintenance	\$8,510,842	\$10,398,245	\$18,909,087
State Hospital Emergency Repairs	\$3,280	\$424,642	\$427,922
TOTAL FYTD 25	\$231,527,321	\$346,962,085	\$578,489,405

List of Acronyms

Acronym	Full Name
A/E	Architecture and Engineering
CMR	Construction Manager At-Risk
COVID-19	Coronavirus Disease 2019
FY	Fiscal Year
FYTD	Fiscal Year to Date
H.B.	House Bill
HHSC	Health and Human Services Commission
HSCS	Health and Specialty Care System
LBHA	Local Behavioral Health Authority
LMHA	Local Mental Health Authority
MSU	Maximum-Security Unit
Non-MSU	Non-Maximum-Security Unit
OFSC	Office of Forensic Services and Coordination
Q	Quarter
S.B.	Senate Bill